

Kushtha Mahakushtha and Kshudrakushtha

Learning Objectives

- ⇒ To learn the definition of Kushtha
- ⇒ Learning the concept of Kushtha
- ⇒ Learning the Nidana Panchaka of Kushtha mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of the disease
- ⇒ To enlist the types of Mahakushtha and Kshudrakushtha
- ⇒ To learn about Purvarupa of Kushtha
- ⇒ To know about Vishishta Lakshana of Mahakushtha and Kshudrakushtha
- ⇒ To know about the Lakshana of Kushtha based on Dosha dominance
- ⇒ To know about the Sadhyasadyata of Mahakushtha and Kshudrakushtha
- ⇒ To know the difference between Mahakushtha and Kshudrakushtha

Introduction

Kushtha is a spectrum of diseases where we can observe symptoms related to integumentary system. Kushtha involves the involvement of all three Dosha and is chronic disease i.e. Chirakari Vikara or slowly manifesting disease. Along with Tridosha, Tvak, Rakta, Mamsa and Ambu are the factors involved in the pathogenesis of the Kushtha.

As per literatures, Kushtha is categorised under Adibala Pravrita Vikara of Adhyatmika Vikara as per classification of Sushruta. Based on the nature of illness, usually Kushtha is considered to be Asadhya Vikara.

Definition of Kushtha

कुष्ठाति शरीरस्थशोणितं विकुरुते इति। Shabda Kalpa Druma

As per Shabda Kalpa Druma, Kushtha is a spectrum of diseases where disturbances in Shonita or Rakta of the individual occurs.

This may result in abnormalities in the functions of Tvak in the form of discolouration, itching, disfiguration or

general symptoms such as raised body temperature or weight loss etc.

Nidana of Kushtha

विरोधीन्यन्नपानानि द्रवस्निग्धगुरुणि च।
भजतामागतां छदिं वेगांश्चान्यान्प्रतिप्रताम्॥4॥
व्यायाममत्तिसंतापमतिभुक्तवोपसेविनाम्।
शीतोष्णालङ्घनाहारान् कर्म मुक्त्या निषेविनाम्॥5॥
धर्मश्रमभयार्तानां दुतं शीताम्बुसेविनाम्।
अजीर्णाभ्यशिनं चैव पञ्चकर्मापचारिणाम्॥6॥
नवाग्रदधिमत्स्यातिलबणाम्लनिषेविनाम्।
मापमूलकपिष्टान्नतिलक्ष्मीरगुडाशिनाम्॥7॥
व्यवायं चाप्यजीर्णेऽने निद्रां च भजतां दिवा।
विप्रान् गुरुन् धर्षयतां पापं कर्म च कुर्वताम्॥8॥

Ca.Ci. – 7/4-8

- ⊕ Excess consumption of incompatible food substances in terms of food and drinks with Drava (liquid), Snigdha (unctuous) and Guru (heavy to digest) qualities¹

When an individual is consuming incompatible food substances, that will lead to inflammation of the stomach as these food substances may remain in the stomach for a period of 2 hours. This will lead to the production of inflammatory markers which are nothing but proteins, and these will lead to the more amount of fluid to get transferred from capillaries of the stomach into the gastric lumen.

- ❖ Suppression of *Chardi* or vomiting or suppression of *Vamana* (emesis) therapy².
- ❖ Excess exercise soon after the consumption of food³.
- ❖ Exposure to sunlight after the consumption of food⁴.
- ❖ Improper consumption of cold and hot substances⁵.
- ❖ Improper following the rituals of fasting and consumption of food⁶.

- ❖ Excess consumption of cold water in cold season⁷.
- ❖ Excess consumption of cold water in extreme physical exertion⁸.
- ❖ Excess consumption of cold water in extreme fearful situation⁹.
- ❖ Excess consumption of food before the digestion of previously ingested meal¹⁰.

- At this particular time period, the ingested food materials which are of macromolecular size, unable to get converted into intermediate or micromolecules due to the compromise in gastric function.
- The inflammation occurred in the gastric lumen will damage the gastric mucosa, the macromolecules which come in contact with damaged gastric mucosa move into the interstitial compartment of the stomach, from there macromolecules enter into the intravascular compartment via lymphatic system. From the intravascular compartment, the macromolecules which act as foreign body in the blood vessels, will enter the interstitial compartment and from there, the entry into interstitial compartment occurs.
- Macromolecules can enter any part of the body. To have the manifestation in the integumentary system, macromolecules will enter the interstitial and intracellular compartments of the skin and appendages.
- Thus, the person may have manifestation of the skin disease.

- 2 Suppression of vomiting can be seen usually in case of the person who is vomiting either because of gastrointestinal or neurological or other non gastrointestinal conditions. When vomiting is suppressed, the substances responsible for vomiting will be transferred from alimentary canal to intravascular compartment and then to interstitial compartment and at last in the intracellular compartment. When the inflammatory markers observed in the interstitial and intracellular compartment. When the disease causing pathogens seen in either interstitial or intracellular compartments, the person may present with *Kushtha* (any skin disease)

When an individual suppresses vomiting, it will lead to tissue injury which may occur as a result of bacteria, trauma or chemicals (medicines). This will lead to the secretion of more amount of chemicals such as histamine, bradykinin etc and these chemicals will enter the intravascular, interstitial and intracellular compartment through a simple process of diffusion. When these chemicals enter into the intracellular compartment of integumentary system, he will present with skin disease.

- 3 Performing excess amount of exercise after the consumption of food, there will be poor blood supply to the alimentary canal and the person have more blood supply to the skeletal muscles. Because of this mechanism, the gastrointestinal secretions will be reduced markedly, thus the person may have digestive impairment.

When an individual is performing exercise soon after the consumption of food, first of all there will poor secretion happen around the alimentary canal leading to poor nutrient absorption and this in turn initiates anaerobic metabolism leading to the accumulation of lactic acid across inside the cells leading to the intracellular accumulation of lactic acid. This accumulation leads to the lowering of the pH of the cell. When the pH is reduced, that will lead to damage of the cell organelles of the cell. If the damage is occurring in the integumentary system, that will lead to the manifestation of skin disease.

- 4 Having excess exposure to sunlight soon after the consumption of food will lead to the increased peripheral circulation of blood deviating the blood to the exterior from the alimentary canal. This will lead to the poor absorption of nutrients, there will be initiation of anaerobic metabolism and thus, there will be accumulation of lactic acid inside the cells leading to the damage of the cells. If the mechanism happens inside the cells of the integumentary system, the person may have the development of the skin disease.

- 5 Improper consumption of cold and hot substances usually responsible for poor and increased secretion of digestive juices in the alimentary canal. This kind of the diet is usually observed in Indians when they visit the restaurants, they consume spicy substances as well as juice or ice cream at a single meal time. This will lead to the poor absorption of nutrients and thus, the body switches over to the anaerobic metabolism when there is abnormal and reduced nutrient absorption leading to the accumulation of lactic acid inside the intracellular compartment. If an individual does fasting for a certain period of time, he should break the fasting by consuming a glass of lemonade and this should be followed by food intake. If the person is having the habit of doing frequent fasting without following appropriate measures of breaking the compartment. Skin disease manifests if there is accumulation of lactic acid in the integumentary system.

- 6 If an individual does fasting for a certain period of time, he should break the fasting by consuming a glass of lemonade and this should be followed by food intake. If the person is having the habit of doing frequent fasting without following appropriate measures of breaking the compartment. Skin disease manifests if there is accumulation of lactic acid in the integumentary system.

- 7 When an individual consumes excess amount of cold water in cold season, first of all, there will be contraction of the secretory cells of mouth, stomach, small intestine. The secretions of these organs are going to get reduced.

- This will reduce gastric juice secretion, the next sequel is hampered food intake and later that will lead to metabolic acidosis (ketoacidosis).
- There is no direct relation with skin disease and cold water consumption in cold season, but one of the major things happens is lowered immune status of the body. Thus, the person is likely to have many of the disease manifestation including skin disease.

- 8 When the person consumes excess amount of cold water in extreme physical exertion, two things are occurring inside the patient: reduced secretion in the alimentary canal compartment and development of metabolic acidosis in the body. This will lead to the reduced food intake, the body gets extreme fatigue secondary to physical exertion.

- 9 Excess consumption of cold water in fearful situation is responsible for sympathetic nervous system activation i.e. reduced alimentary secretion and contraction of sphincters of alimentary canal. This may lead to the poor digestive activity in the digestive system.

- This will initially lead to the utilisation of deposited nutrients in the form of sequential use of carbohydrates, fats and proteins. Thus, the person may attain metabolic acidosis.
- This is termed as *Adhyashana*. When this happens in an individual, following events happen in the human body.
- Closure of pyloric antrum immediately after consuming excess diet for a period of 2 hours.

- Improper post Panchakarma diet and regimen¹¹.
- Excess consumption of freshly harvested grains¹².
- Excess consumption of curds¹³.
- Excess consumption of fish¹⁴.
- Excess consumption of Lavana (salt) and Amla (sour) Rasa substances¹⁵.
- Excess consumption of blackgram, radish, flour items, sesaunum seeds, milk and jaggery¹⁶.
- Excess sexual intercourse in indigested state¹⁷.
- Excess sleep during day time^{18,19}.
- Not giving appropriate respect to elders and teachers²⁰.
- Performing Papa Karma (ill deeds) done in one's previous or present life.

Samprapti of Kushtha

वातादयस्त्रयो दुष्टास्त्वग्रतं मांसमम्बु च।
दृष्यन्ति स कुष्ठानां सप्तको द्रव्यसंग्रहः॥१९॥
अतः कुष्ठानि जायन्ते सप्त चैकादशैव च।

न चैकदोषजं किञ्चित् कुष्ठं समुपलभ्यते॥१०॥

Ca.Ci. - 7/9,10

- Due to above said etiological factors, Vata, Pitta and Kapha Dosha get vitiated.
- These vitiated Dosha will afflict Tvak, Rakta, Mamsa and Ambu.
- The Tridosha and above four factors involved in the disease Kushtha are together termed as Saptaka Dravya Sangraha.
- Altogether, the above seven factors are responsible for the manifestation of Kushtha where vitiated Dosha from the Koshttha move to the Shakha, thereby responsible for the manifestation of Kushtha.
- All Kushtha are invariably considered to be Tridoshaja by nature. Ayurveda literatures described 7 types of Mahakushtha and 11 types of Kshudrakushtha.

Samprapti Ghataka

- Dosha : Tridosha
- Dushya : Rasa, Rakta, Mamsa, Ambu
- Srotas : Rasavaha, Raktavaha, Mamsavaha and Udakavaha Srotas.

- Macromolecules of recently consumed food pave the way to move earlier consumed macromolecules or intermediate molecules to the stomach wall.
- Poor functioning of gastric juice: pH of gastric juice >5; inactivation of pepsinogen into pepsin, proliferation of bacteria in the stomach as carbohydrate is most of the occasions, major diet consumed frequently.
- Inflammation of the stomach happens, leading to the formation of inflammatory markers, which leads to more secretion from the stomach into the gastric lumen.
- This leads to the gastric mucosal damage. This will lead to the transport of macromolecules and intermediate molecules from the gastric lumen into the interstitial compartment of stomach to the lymphatics, thence to the superior vena cava at the junction between left internal jugular vein and left subclavian vein.
- As these are macromolecules in nature, these substances are not utilised by the body. Thus, body tries to utilise the fats and thus leading to metabolic acidosis (ketoacidosis and lactic acidosis).
- 11 Improper post Panchakarma diet and regimen refers to not following the advices given by the Panchakarma expert. This may be in the form of consuming curds after 1st day of Virechana or Vamana; travelling long distance of more than 100 kms or more or speaking out loud in a gathering or travelling in uneven roads or exposed to dust or smoke.
 - In the context of Kushtha, exposure to dust or smoke or cold air contact to the skin can be the risk factor for the manifestation of skin disease.
- 12 As per Ayurveda literatures, freshly harvested grains will lead to abnormal increase of Kapha Dosha in the body, if the person is susceptible, may be responsible for Kushtha.
- 13 Excess consumption of curds will lead to the vitiation of Kapha and Pitta Dosha, either single Dosha or both Dosha depending upon the composition of curds. If the skin is susceptible, the person may develop the manifestation of Kushtha.
- 14 As per Ayurveda literature, the qualities of fish are Madhura Rasa, Guru and Snigdha qualities, Ushna Virya and Madhura Vipaka. Excess consumption of fishes is responsible for the abnormal increase of either Pitta or Kapha Dosha or both. If the skin is susceptible, vitiated Dosha will lead to manifestation of Kushtha in the individual.
- 15 Excess consumption of Lavana and Amla Rasa will lead to abnormal increase of Pitta or Kapha or both Dosha in the body, thus may lead to the manifestation of Kushtha, if he is susceptible.
- 16 By consuming above said substances, there will be vitiation of Kapha Dosha in the body.
- 17 Excess sexual intercourse in indigested state, leads to the increased basal metabolic rate; sequential depletion of carbohydrate, fat and protein in the body, thus lowering immune status of the body.
- 18 Excess indulgence in sleep during day time will lead to the increase of Kapha Dosha in the body. If Khavaigunya is present in Tvak and if the individual is sleeping excessively during day time, he may be presented with Kushtha.
- 19 This will lead to the extreme vitiation of Kapha Dosha instantly.
- 20 To understand this concept, we have to remember the formula M-TFAR-D. As per this formula, your destiny is decided by your mindset which influences your thoughts, feelings, actions and results.
 - Elders and teachers will give the advices based on the experiences in terms of healthy lifestyle.
 - Not giving the due consideration to elders, leads to non-following of advices, thus the person may have multiple diseases including skin disease.
 - Refer the context of Unmada for this to understand even better.

- ❖ **Srotodushti Prakara:** *Sanga and Vimargagamana.*
- ❖ **Agni :** *Jatharagnimandya, Rasadhatvagni and Raktadhatvagnimandya.*
- ❖ **Ama :** *Jatharagnijanya, Rasadhatvagnijanya and Raktadhatvagnijanya*
- ❖ **Udbhava Sthana:** *Either Amashaya or Pakvashaya depending upon the initial Dosha vitiation.*
- ❖ **Sanchara Sthana :** *Sarva Sharira.*
- ❖ **Vyakta Sthana :** *Tvak.*
- ❖ **Rogamarga :** *Abhyantara, Bahya.*
- ❖ **Sadhyasadhya :** *Asadhya.*
- ❖ **Vyadhyavastha:** *Chirakari.*

Purvarupa of Kushtha²¹

स्पर्शाशत्वमतिखेदो न वा वैषण्यमुन्नतिः।

कोठानां लोमहर्षश्च कण्डूस्तोदः श्रमः क्रमः॥1 1॥

प्रणानामधिकं शूलं शीघ्रोत्पत्तिधिरस्थितिः।

दाहः सुताङ्गता चेति कुष्ठलक्षणमग्रजम्॥1 2॥

- ❖ **Numbness²².**
- ❖ **Excess sweating²³.**
- ❖ Little or no sweating at all.
- ❖ Discolouration of the skin.
- ❖ Papular skin lesions.
- ❖ Skin eruptions.
- ❖ Horripulation.
- ❖ Itching.
- ❖ **Pricking type of pain²⁴.**
- ❖ **Giddiness²⁵.**
- ❖ **Fatigue without any physical exertion²⁶.**
- ❖ Formation of multiple ulcers and persistence of ulcers for long time.
- ❖ Burning sensation.
- ❖ **Tingling sensation²⁷**

G
K

- 21 This is a disease afflicting Tvak (skin and its appendages). When we speak of skin, it contains 5 different types of receptors.
 1. Mechanical receptors which detect the mechanical compression or stretching
 2. Thermoreceptors detect the changes in the temperature either cold or warmth
 3. Nociceptors – which detect damage occurring in the tissues also termed as pain receptors
 3. Electromagnetic receptors – detect light on the retina of the eye
 5. Chemoreceptors – detect taste in the mouth, smell in the nose, oxygen level in the arterial blood, carbon dioxide concentration in the body etc.

Among these types, mechanoreceptors, thermoreceptors and nociceptors are of significance as far as skin disease is concerned.
- 22 Skin is having different receptors for the conduction of different types of sensations. Tactile sensation, temperature, pressure, vibration, pain, itching etc are carried by different types of receptors. When an individual is having different skin diseases, this may lead to damage of different sensory receptors. Depending upon the nature of damage to sensory receptors, the person may have different abnormal sensory perception including numbness or temperature imperception or pressure sensation etc.
- 23 Two types of sweat glands are seen – apocrine sweat glands and eccrine sweat glands depending upon the structure and type of secretion. There are 3 to 4 million sweat glands are present in the human body.
 - Eccrine sweat glands are distributed throughout the skin of most regions of the body, especially in the skin of forehead, palms and soles. These glands are not present in margins of lips, nail beds of fingers, glans penis, glans clitoris, labia minora or eardrums. The secretory portion of eccrine sweat glands is located mostly in deep dermis. The excretory portion projects through dermis and epidermis and ends as a pore at the surface of the epidermis.
 - o This leads to the production of sweat which contains more amount of water, with small amounts of ions sodium and chloride, urea, uric acid, ammonia, amino acids, glucose, and lactic acid.
 - o This also helps in regulation of body temperature.
 - o These sweat glands also release sweat in response to emotional stress which is referred as emotional sweating or cold sweat.
 - Apocrine sweat glands are simple coiled tubular glands, but have larger ducts and lumens than eccrine glands. They are found in the skin of axilla, groin, areolae of breast and bearded regions of face in adult males.
 - o Compared to eccrine sweat, apocrine sweat appears milky or yellowish in colour. The components are similar to eccrine sweat plus lipids and proteins. Apocrine sweat is odourless.
 - o Apocrine sweat glands produce sweat during sexual activities.

In case of skin disease, there may be the damage to the dermis or epidermis, thus damage to the sweat glands may occur leading to hampering of sweat production.

 - Increase in sweat production may occur as a result of increased peripheral circulation which occurs to combat the pathogen in the skin.
- 24 Depending upon the nature of exposure of the pathogen to the part of the body or whole body, the person may develop either localised or patchy or widespread skin lesions observed in the patient. First of all, the clinician has to decide whether these lesions are primary or secondary in appearance. These are helpful in the differential diagnosis of the skin diseases.
- 25 The presence of giddiness implies the person is receiving less amount of blood supply to the brain which is termed as syncopal episodes. This occurs due to the reshift of the blood to the peripheral circulation secondary to the development of skin lesions.
- 26 As far as Ayurveda fundamentals are concerned, Kushtha is a condition which has a long-standing history (Chirakari) disease. Due to this, the person may have sequential depletion of carbohydrate, fats and proteins. Thus, the person may have fatigue without doing any strenuous physical activity.
- 27 These symptoms are seen as a result of abnormal sensory perception. This can result from damage to the sensory receptors of the integumentary system.

Types of Kushtha

अत ऊर्ध्वमष्टादशानां कुष्ठानां कपालोदुम्बरमण्डलजिह्वपुण्डरीकसिमका
कण कैककुष्ठचर्मस्यकिटिमविपादिकालसकदचर्मदलपामाविस्कोट-
कशतारुर्विचर्चिकानां लक्षणाम्युपदेक्ष्यामः॥13॥ Ca.Ci. – 7/13

7 Mahakushtha are Kapala, Udumbara, Mandala, Rishyajiiva, Pundarika, Sidhma and Kakanaka.

11 Kshudrakushtha are Ekakushtha, Charmakhya, Kitibha, Vipadika, Alasaka, Dadru, Charmadala, Pama, Visphotaka, Shataru, Vicharchika.

Mahakushtha: as per Charaka

S.No.	Mahakushtha	Dosha dominance
1.	Kapala	Vata
2.	Udumbara	Pitta
3.	Mandala	Kapha
4.	Rishyajiiva	Vata + Pitta
5.	Pundarika	Kapha + Pitta
6.	Sidhma	Vata + Kapha
7.	Kakanaka	Vata + Pitta + Kapha

Kshudrakushtha: as per Charaka

S.No.	Kshudrakushtha	Dosha dominance
1.	Ekakushtha	Vata + Kapha
2.	Charmakhya	Vata + Kapha
3.	Kitibha	Vata + Kapha
4.	Vipadika	Vata + Kapha
5.	Alasaka	Vata + Kapha
6.	Pama	Pitta + Kapha
7.	Shataru	Pitta + Kapha
8.	Visphota	Pitta + Kapha
9.	Dadru	Pitta + Kapha
10.	Charmadala	Pitta + Kapha
11.	Vicharchika	Kapha

Kapala

कृष्णारुणकपालाभं यदूक्षं परुषं तनु।

कपालं तोदवडलं तत्कुष्ठं विषमं स्मृतम्॥14॥

Ca.Ci. – 7/14

- ✿ The skin lesion appears like a piece of broken earthen vessel.
- ✿ Blackish or reddish coloured skin lesion
- ✿ Dry or rough in appearance
- ✿ Thin in texture

- ✿ Very severe pricking type of pain



Figure 88: Vata dominant lesion

Udumbara

दाहकण्डूरुजारागपरीतं लोमपिञ्जरम्।

उदुम्बरफलाभासं कुष्ठमौदुम्बरं विदुः॥15॥

Ca.Ci. – 7/15

- ✿ The appearance of skin lesion resembles fruit of Udumbara bright yellow in the centre and red around yellow discolouration
- ✿ Burning sensation around skin lesions
- ✿ Itching sensation around the skin lesion
- ✿ Severe pain around the skin lesion
- ✿ Redness around the skin lesion
- ✿ Matted body hairs around the skin lesion



Figure 89: Pitta dominant lesion

Mandala

श्वेतं रक्तं स्थिरं स्त्यानं स्निग्धमुत्सन्नमण्डलम्।

कृच्छ्रमन्योन्यसंसक्तं कुष्ठं मण्डलमुच्यते॥16॥

Ca.Ci. - 7/16

- Whitish discolouration of the skin lesion
- Reddish discolouration of the skin lesion
- Thick, circular, firm skin lesion
- Slimy or wet appearance
- Difficult to get cure from the skin lesion



Figure 90: Kapha dominant lesion

Rshyajihva



Figure 91: Vata and Pitta dominance

कर्कशं रक्तपर्यन्तमन्तः श्यावं सवेदनम्।

यदृष्यजिह्वासंस्थानमृष्यजिह्वं तदुच्यते॥17॥

Ca.Ci. - 7/17

- The skin lesion is rough in appearance
- Reddish discolouration throughout the skin lesion and blackish discolouration inside the skin lesion
- Painful lesion
- The appearance of skin lesion resembles the appearance of tongue of deer

Pundarika

सश्वेतं रक्तपर्यन्तं पुण्डरीकदलोपमम्।

सोत्सेधं च सदाहं च पुण्डरीकं तदुच्यते॥18॥

Ca.Ci. - 7/18

- Reddish white skin lesion just like petals of lotus flower
- Elevated skin lesion
- Associated with burning sensation



Figure 92: Pitta and Kapha dominance

Sidhma



Figure 93: Vata and Kapha dominance

श्वेतं तानं तनु च यद्वैजो घृष्टं विमुञ्चति।

अलाबूपुष्पवर्णं तत् सिध्मं प्रायेण चोरसि॥19॥

Ca.Ci. - 7/19

- Coppery white discolouration of the skin lesion
- On itching, there will be shedding of thin, white, coppery powder from the skin
- The skin lesion resembles the colour of flower of *Alabu*.
- The appearance of skin lesion is usually seen in the thoracic region

Kakanaka

यत् काकणान्तिकावर्णमपाकं तीव्रवेदनम्।

त्रिदोषलिङ्गं तत् कुष्ठं काकणं नैव सिध्यति॥20॥

Ca.Ci. - 7/20

- The skin lesions resemble the seed of *Gunja* i.e. black in the centre surrounded by reddish discolouration of the skin lesion



Figure 94: Tridosha dominance

- ❖ There will not be suppuration in the skin lesion
- ❖ It is associated with severe pain
- ❖ There will be vitiation of all three *Dosha*
- ❖ The skin lesions are incurable

Kshudrakushtha

Ekakushtha



Figure 95: Lakshana of Ekakushtha

Psoriasis which is widespread even seen in scalp region
अस्वेदनं महावास्तु यन्मत्स्यशकलोपमम्।

तदेककुष्ठं,

Ca.Ci. – 7/21

- ❖ Absence of sweating in the area of skin lesion
- ❖ Affects large area of skin, widespread and diffuse skin lesion
- ❖ The appearance of skin is like skin of fish

Charmakhya

चर्माख्यं बहलं हस्तिचमेवत्॥२१॥

Ca.Ci. – 7/21

- ❖ The skin lesions are usually thick and hard like skin of the elephant.

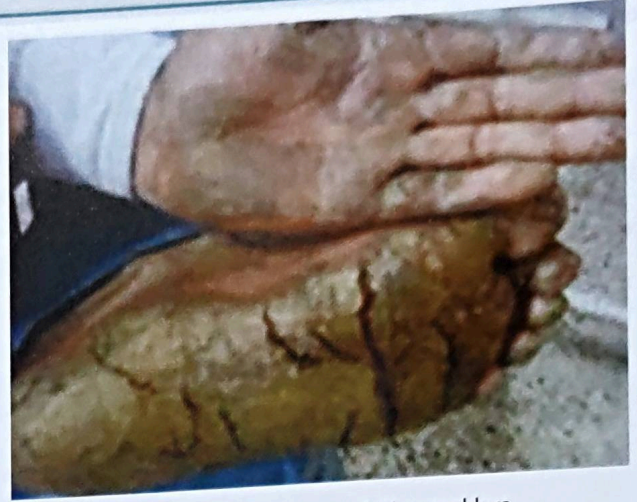


Figure 96: Lakshana of Charmakhya

Kitibha



Figure 97: Lakshana of Kitibha

श्यावं किणखर स्पर्श परुषं किटिमं स्मृतम्।

Ca.Ci. – 7/22

- ❖ Blackish skin lesion
- ❖ Dry and rough skin lesion similar to the appearance and texture of healed wound

Vaipadika



Figure 98: Lakshana of Vaipadika

वैपादिकं पाणिपादस्फुटनं तीव्रवेदनम्॥2 2॥

Ca.Ci. - 7/22

- ✧ Cracks in palms and soles of the individual
- ✧ Associated with severe pain

Alasaka

कण्डूमद्भिः सरागैश्च गण्डैरलसकं चितम्।

Ca.Ci. - 7/23

- ✧ The skin lesions are red
- ✧ Profuse itching



Figure 99: Lakshana of Alasaka

Dadru



Figure 100: Lakshana of Dadru

सकण्डूरागपिडकं दद्रुमण्डलमुद्गतम्॥2 3॥

Ca.Ci. - 7/23

- ✧ Circular lesion
- ✧ Small reddish lesion
- ✧ Associated with itching

Charmadala

रक्तं सकण्डु सस्फोटं सरुग्दलति चापि यत्।

तच्चर्मदलमाख्यातं संस्पर्शासहमुच्यते॥2 4॥

Ca.Ci. - 7/24

- ✧ Reddish lesion
- ✧ Itching
- ✧ Papular skin eruption
- ✧ Painful skin eruption
- ✧ Appearance of fissure in the skin eruption
- ✧ Tenderness in the skin eruption



Figure 101: Lakshana of Charmadala

Pama



Figure 102: Lakshana of Pama

पामा श्वेतारुणश्यावाः कण्डूलाः पिडका भृशम्।

Ca.Ci. - 7/25

- ✧ Whitish, reddish or blackish skin eruption
- ✧ Associated with itching

Visphota



Figure 103: Lakshana of Visphota

स्फोटाः श्वेतारुणाभासो विस्फोटाः स्युस्तनुत्वचः॥

- ❖ Whitish or reddish skin eruptions
- ❖ Thin skin eruptions

Ca.Ci. – 7/25

Shataru

रक्तं श्यावं सदाहर्ति शताः स्याद्बहुव्रणम्।

- ❖ Reddish or blackish skin eruptions
- ❖ Burning sensation
- ❖ Painful skin eruption
- ❖ Multiple wound formation

Ca.Ci. – 7/26



Figure 104: Lakshana of Shataru

Vicharchika



Figure 105: Lakshana of Vicharchika

सकण्डूः पिडका श्यावा बहुस्रावा विचर्चिका॥

- ❖ Blackish skin eruption
- ❖ Associated with itching
- ❖ Excess oozing from the skin lesion

Ca.Ci. – 7/26

Dosha dominance and Lakshana of Kushtha

रौक्ष्यं शोषस्तोदः शूलं संकोचनं तथाऽऽयामः।

पारुष्यं खरभावो हर्षः श्यावारुणत्वं च॥3 4॥

कुष्ठेषु वातलिङ्गं, दाहो रागः परिस्त्रवः पाकः।

विस्त्रो गन्धः क्वेदस्तथाऽङ्गपतनं च पित्तकृतम्॥3 5॥

शैत्यं शैत्यं कण्डूः स्थैर्यं चोत्सेध गौरव स्नेहाः।

कुष्ठेषु तु कफलिङ्गं जन्तुभिरभिभक्षणं क्लेदः॥3 6॥

Ca.Ci. – 7/34-36

As all the varieties of *Kushtha* are *Tridoshaja* in manifestation, it will be difficult to identify the clinical features of *Kushtha*. That is why specific *Lakshana* in relation to *Dosha* vitiation are specifically mentioned in *Ayurveda* literatures.

Vataja Kushtha Lakshana:

1. Dryness of the skin (see Figure 106)
2. Shrinkage of the skin (see Figure 107)
3. Pricking type of pain
4. Contracture of the skin (see Figure 108)
5. Bending of the body or deformities in body parts (see Figure 109)
6. Roughness of the skin (see Figure 110)
7. Horripulation
8. Blackish or reddish discolouration (see Figure 111 and 112)

Pittaja Kushtha Lakshana:

1. Burning sensation of the skin
2. Redness of the skin (see Figure 113)
3. Secretion from the skin lesion (see Figure 114)
4. Suppuration or pus formation (see Figure 115)
5. Foul smell from the skin lesion
6. Increased moisture
7. Shedding of the tissues from the wound (sloughing) (see Figure 116)

Kaphaja Kushtha Lakshana

1. Whitish discolouration of the skin lesion
2. Cold in touch
3. Itching
4. Firmness in the skin lesion
5. Heaviness of the skin lesion
6. Unctuousness of the skin lesion
7. Appearance of maggots in the wound eating the skin and surrounding tissues (see Figure 117)

Thus, *Dosha* dominance in *Kushtha* spectrum of diseases is determined by either variations in colour of the lesion or/ and morphology of the lesions or/ and clinical features of the disease or/ and site of the lesions.



Figure 106: Rukshata or dryness of the skin



Figure 107: Shosha or shrinkage of the skin



Figure 108: Sankocha or contracture of skin



Figure 109: Angahinata or deformities of the body



Figure 110: Kharatva or roughness of the skin



Figure 111: Shyava Varna or blackish discolouration



Figure 112: Aruna Varna or reddish discolouration



Figure 113: Rakta Varna or Raga



Figure 114: Puyasrava or discharge



Figure 115: Putisrava or pus discharge



Figure 116: Kotha or sloughing



Figure 117: Appearance of Krimi

Dhatugata Kushtha

स्पर्शहानिः स्वेदनत्वमीषत्कण्डूश्च जायते।
 वैवर्ण्यं रूक्षभावश्च कुष्ठे त्वचि समाश्रिते॥2 2॥
 त्वकस्वापो रोमहर्षश्च स्वेदस्याभिप्रवर्तनम्।
 कण्डूर्विपूयकश्चैव कुष्ठे शोणितसंश्रिते॥2 3॥
 बाहुल्यं वक्रशोषश्च कार्कश्यं पिडकोद्गमः।
 तोदः स्फोटः स्थिरत्वं च कुष्ठे मांससमाश्रिते॥2 4॥
 दौर्गन्ध्यमुपदेहश्च पूयोऽथ क्रिमयस्तथा।
 गात्राणां भेदनं चापि कुष्ठे मेदः समाश्रिते॥2 5॥
 नासाभङ्गोऽक्षिरागश्च क्षते च क्रिमिसंभवः।
 भवेत् खरोपघातश्च अस्थिमज्जसमाश्रिते॥2 6॥
 कौण्यं गतिक्षयोऽङ्गानां संभेदः क्षतसर्पणम्।
 शुक्रस्थानगते लिङ्गं प्रागुक्तानि तथैव च॥2 7॥

Su.Ni. – 5/22-27

- ❖ **Tvak Gata Kushtha:** Reduced touch perception, sweating, mild itching, discolouration, dryness of the skin lesion
- ❖ **Rakta Gata Kushtha:** Absence of sensation in the skin, horripulation, sweating, itching, pus formation
- ❖ **Mamsa Gata Kushtha:** Multiple skin lesions, dryness of the mouth, roughness of the skin lesion, small and firm eruptions associated with pricking type of pain

- ❖ **Medo Gata Kushtha:** foul smell from the skin lesion due to discharge, pus formation and discharge, formation of maggots, cracks in different body parts.
- ❖ **Asthi Gata Kushtha:** Loss of nasal bridge, redness of the eyes, appearance of maggots in wound in the skin, hoarseness of voice.
- ❖ **Majja Gata Kushtha:** Similar *Lakshana* of *Asthi Gata Kushtha* are observed.
- ❖ **Shukra Gata Kushtha:** smell of dead body from the skin lesion, reduced movement in the patient who has this type of *Kushtha*; deformities of body, spread of wound and all other *Lakshana* mentioned in earlier varieties of *Kushtha*.

Upadrava of Kushtha

अस्यां चैवावस्थायामुपद्रवाः कुष्ठिनं स्पृशन्ति; तद्यथा - प्रस्त्रवणमङ्गभेदः पतनान्यङ्गावयवानां तृष्णाज्व- रातीसारदाहदौर्बल्यारोचकाविपाकाश्च, तथाविधमसाध्यं विद्यादिति॥1१॥

Ca.Ni. - 5/11

- ❖ **Oozing**²⁸ from the wound.
- ❖ **Shedding of tissues**²⁹ from the body parts.
- ❖ **Shedding of body parts**³⁰ from the body.
- ❖ **Thirst**³¹.
- ❖ **Raised body temperature**³².

- ❖ **Diarrhoea**³³.
- ❖ **Burning sensation**³⁴.
- ❖ **Weakness**³⁵.
- ❖ **Tastelessness**.
- ❖ **Impaired digestion**³⁶.

Sadhyasadhya of Kushtha

सर्वैर्लिङ्गैर्युक्तं मतिमान् विवर्जयेदवलम्।
तृष्णादाहपरीतं शान्ताग्निं जन्तुभिर्जग्धम्॥3७॥
वातकफप्रबलं यद्यदेकदोषोल्बणं न तत् कृच्छ्रम्।
कफपित्त वातपित्तप्रबलानि तु कृच्छ्रसाध्यानि॥3८॥

Ca.Ci. - 7/37,38

साध्यं त्वग्रक्तमांसस्थं वातश्लेष्माधिकं च यत्।
मेदसि द्वन्द्वजं याप्यं वर्ज्यं मज्जास्थिसंश्रितम्॥3९॥
क्रिमिवृद्धाहमन्दाग्निसंयुक्तं यन्निदोषजम्।
प्रभिन्नं प्रस्तुताङ्गं च रक्तनेत्रं हतस्वरम्॥3९॥
पञ्चकर्मगुणातीतं कुष्ठं हन्तीह मानवम्।

Ma.Ni. - 49/31-33

- ❖ When all *Lakshana*³⁷ mentioned in *Kushtha* are seen in the patient.
- ❖ If *Rogibala*³⁸ (strength of the patient) is poor than *Rogabala* (strength of the disease).
- ❖ Associated with thirst and **burning sensation**³⁹.

- 28 When an individual is suffering from skin lesions filled with fluid (vesicles or pustules), itching in such occasion, may lead to oozing of the fluid or pus from the wound or ulcer.
 - Oozing from the ulcer may occur in trivial disease such as eczema or serious illness such as pemphigus vulgaris.
 - Pus can be observed in cellulitis etc
- 29 Shedding of the tissues from the body parts can be observed in conditions such as psoriasis or dry eczema or atopic dermatitis or contact dermatitis etc.
- 30 This may be observed in complications of leprosy where in the person may lose the part of the body due to inability to feel the sensation or temperature. Repeated damages to the affected parts may lead to loss of digits or fingers etc.
- 31 The presence of thirst indicates shrinkage of intravascular compartment. In the person suffering from skin disease, which releases inflammatory markers in the interstitial compartment which may lead to attraction of WBCs etc to the interstitial compartment from the blood vessels. If the lesions are widespread, then there is a possibility of shrinking of intravascular compartment which may lead to appearance of thirst.
 - Thirst may also be seen in the patients suffering from skin infections along with diabetes mellitus. In this situation, skin infection will not come under control unless high blood sugar level is being checked. In this situation, thirst may manifest because of polyuria.
- 32 The presence of raised body temperature may imply the ongoing infective pathology inside the patient. E.g. in cellulitis or pyodermitis conditions, the person may develop fever.
- 33 This may be observed as a result of adverse drug reaction.
- 34 Burning sensation may be observed in the patient who is having peripheral neuropathy.
- 35 In case of person who is suffering from chronic skin disease, the person may have sequential depletion of nutrients in the form of carbohydrate, fat and protein respectively. If the person is taking very little amount of food, that may lead to weakness.
- 36 When an individual is suffering from pemphigus vulgaris, he or she may have initial involvement of oral cavity, thereby he may have either tastelessness (due to poor oral secretion) and later he may have digestive impairment (due to impaired alimentary canal secretion which may occur as a result of autonomic nervous dysfunction).
- 37 All the *Lakshana* present in *Kushtha* indicates the severity of the illness. For example, the skin lesion present in right palmar surface of hand is much less severe than the widespread skin lesion observed in diffused way in the skin.
 - If the skin lesion is associated with fever indicates much severe *Lakshana*
 Thus, all *Lakshana* mentioned in *Kushtha* may imply severity of the illness.
- 38 Whenever we analyse the patients, we have to take two points into consideration – *Rogibala* and *Rogabala*. When the *Rogibala* is more than the severity of *Rogabala*, then we can administer much potent medicine, so that the patient can withstand the potency of medicine. If the *Rogibala* is much less than *Rogabala*, then the clinician has to use milder form of medication, so that little discomfort to the patient can be ensued.
- 39 The presence of thirst implies that the person is having shrinking intravascular compartment. Indirectly, this may imply that shrinking of interstitial and intracellular compartment.

- ❖ Loss of appetite⁴⁰.
- ❖ Appearance of maggots⁴¹ in the wounds.
- ❖ Kushtha having the dominance of Vata and Kapha, Kushtha having single Dosha vitiation are considered to be Sadhya (easily curable)⁴².
- ❖ Kushtha with the vitiation of Kapha and Pitta, Vata and Pitta is Krichrasadhyā (difficult to cure)⁴³.
- ❖ Tvak Gata, Rakta Gata, Mamsa Gata Kushtha, Vata and Kapha dominant Kushtha is considered to be Sadhya (easy to cure)⁴⁴.
- ❖ Medo Gata Kushtha, Pitta and Kapha dominant, Vata and Pitta dominant Kushtha is considered to be Yasya (manageable).
- ❖ Asthi Gata Kushtha and Majja Gata Kushtha, Kushtha associated with appearance of maggots, thirst, burning sensation, loss of appetite, Tridoshaja Kushtha, separation of body parts from body due to Kushtha, reddish discolouration of the eyes, loss of voice and Kushtha beyond the control of Panchakarma treatment are considered to be Asadhya (incurable).

Differences between Mahakushtha and Kshudrakushtha

Mahakushtha	Kshudrakushtha
Deeper Dhatu affliction seen	Deeper Dhatu affliction not seen
Associated with Purvarupa such as Tvak Sankocha, Tvak Bheda, Tvak Achetantva and Angasada	Not associated with Purvarupa as mentioned in the context of Mahakushtha
Severe degree of Dosha vitiation is seen	Mild or moderate degree of Dosha vitiation is seen
Widespread affliction of skin is seen	Localised or patchy distribution of skin lesions are seen
Rogabala is more than Rogibala in most occasions	Rogibala is more than Rogabala

Key summary of the chapter

कुष्माति शरीरस्थशोणितं विकुरुते इति।

Shabda Kalpa Druma

- ❖ As per Shabda Kalpa Druma, Kushtha is a spectrum of diseases where disturbances in Shonita or Rakta of the individual occurs. In most occasions, Tridosha, Tvak, Rakta, Mamsa and Ambu are involved during pathological events of Kushtha and these seven factors are considered as Saptaka Dravya Sangraha. This is considered as Chirakari Vikara. As per Sushruta, this is categorised under Adibala Pravritta Vikara of Adhyatmika Vikara. As far as Udbhava Sthana is concerned, Amashaya is the Udbhava Sthana in case of Pitta and Kapha dominant Kushtha, whereas Pakvashaya is considered as Udbhava Sthana Vata dominant Kushtha. Sanchara Sthana of Kushtha is Sarva Sharira and Vyakta Sthana is Tvak.

स्यशांशत्यमतिलेदो न या देवण्यमुप्रतिः। कोठानां लोमहर्षच कण्डूस्तोदः अमः क्रमः॥1१॥

प्रणानामधिकं शूलं शीघ्रोत्पत्तिधिरस्थितिः। बाहः सुताङ्गता चेति कुष्ठलक्षणमग्रजम्॥ 12॥

Ca.Ci. – 7/11-12

- ❖ As far as Purvarupa are concerned, most of the Purvarupa are related to the abnormal structure or functioning of Tvak. Shrama, Klama, Daha, Suptangata are subjective manifestations of Purvarupa.
- ❖ Sankhya Samprapti of Kushtha: 7 types of Mahakushtha and 18 types of Kushtha which include 7 types of Mahakushtha and 11 types of Kshudrakushtha. 7 types of Mahakushtha are Kapala, Udumbara, Mandala, Rshyajihva, Pundarika, Sidhma and Kakanaka Kushtha. 11 types of Kshudrakushtha are Ekakushtha, Charmakhya, Kitibha, Vipadika, Alasaka, Charmadala, Pama, Visphota, Dadru, Shataru, Vicharchika.
- ❖ Sushruta has mentioned 7 types of Dhatugata Kushtha. These are indicative of severity of the disease.
- ❖ Duration of the illness, nature of involvement of Dhatu, presence or absence of Purvarupa in the patients of Kushtha determine the diagnosis of Mahakushtha and Kshudrakushtha.

- The presence of burning sensation implies the destruction of sensory receptors of skin, particularly temperature sensing receptors.
- 40 Loss of appetite implies poor digestive fire. It indicates the person is unable to digest the ingested food and unable to digest the medicines too. In other words, neither the nutrients nor the medicines can cross the alimentary canal into intravascular compartment, thence to interstitial and at last to intracellular compartments.
- 41 Maggot infestation is a condition in which the fly maggots feed off and develop in the tissues of living organisms. True myiasis results from flies deliberately laying eggs in or on the tissues. This may delay the wound healing.
- 42 The Guna common to Vata and Kapha Dosha is Shita Guna. That means when we plan the treatment it has to be Ushna line of treatment has to be administered. Ekadoshaja diseases are considered to be treatable by default as per Ayurveda experts.
- 43 This may imply the severity of the disease as Two Dosha vitiation is observed in the individual.
- 44 All these are indicative of nature of affliction of body tissue, multitude of Dosha affliction, association of other symptoms and superficial or deep seatedness of the disease.

Self-Assessment: Questions for Reviews

Long essay questions

- Define Kushtha, explain the Nidana, Samprapti, Purvarupa, Rupa, Bheda, Upadrava, Sadhyasadyata of Kushtha
- Explain Kushtha Nidana Panchaka in detail
- Explain Mahakushtha in detail
- Explain Kshudrakushtha in detail

Short essay questions

- Explain Sidhma Kushtha in detail
- Explain Kshudrakushtha in brief
- Explain the features of Vata Pradhana Kushtha

MCQs

1. As per Charaka, Kapha dominant Kushtha is

- a. Kapala; b. Udumbara;
- c. Mandala; d. Pundarika

Answer: (c)

2. As per Charaka, the number of Kshudrakushtha is

- a. 7; b. 11;
- c. 18; d. Innumerable

Answer: (b)

3. Which of the following is not included in Saptaka Dravya Sangraha?

- a. Tvak; b. Rakta;
- c. Mamsa; d. Asthi

Answer: (d)

4. Which of the following is termed as Hansen's disease?

- a. SLE; b. Leprosy;
- c. Tuberculosis; d. Syphilis

Answer: (b)

5. Psoriasis is

- a. Bacterial disease;
- b. Viral disease;
- c. Parasitic disease;
- d. Non infective disease

Answer: (d)

6. Silvery scale is characteristic of which of the following disease?

- a. Tinea vulgaris;
- b. Sycosis barbae;
- c. Psoriasis;
- d. Bilharziasis

Answer: (c)

7. Which of the following is included in Mahakushtha?

- a. Ekakushtha; b. Kitibha;
- c. Pundarika; d. Vipadika

Answer: (c)

8. Dosha dominance observed in Sidhma Kushtha is

- a. Vata and Pitta;
- b. Pitta and Kapha;
- c. Vata, Pitta and Kapha;
- d. Vata and Kapha

Answer: (d)

9. Udumbara Kushtha is having the dominance of

- a. Vata; b. Pitta;
- c. Kapha; d. Vata and Kapha

Answer: (b)

10. As per Sushruta, Kushtha is categorised under

- a. Janmabala Pravritta Vikara;
- b. Sanghatabala Pravritta Vikara;
- c. Svabhavabala Pravritta Vikara;
- d. Adibala Pravritta Vikara

Answer: (d)

11. Mahavastu is the specific Lakshana observed in which of the following type of Kushtha?

- a. Shataru; b. Vicharchika;
- c. Charmakhya; d. Ekakushtha

Answer: (d)

12. The skin resembles the skin of the elephant in which of the following?

- a. Ekakushtha; b. Charmakhya;
- c. Vipadika; d. Vicharchika

Answer: (b)

13. Which of the following Kshudrakushtha is having the dominance of Kapha Dosha?

- a. Vicharchika; b. Vipadika;
- c. Visphota; d. Kitibha

Answer: (a)

14. Sankhya Samprapti of Mahakushtha is

- a. 7; b. 11;
- c. 18; d. Innumerable

Answer: (a)

15. The colour of skin lesion of Sidhma resembles which of the following colour?

- a. Durva; b. Alabu;
- c. Atasi; d. Jambu

Answer: (b)

Chapter resources

- ✿ Charaka Samhita Nidana Sthana – 5th chapter – Kushtha Nidana Adhyaya
- ✿ Charaka Samhita Chikitsa Sthana – 7th chapter – Kushtha Chikitsa Adhyaya
- ✿ Sushruta Samhita Nidana Sthana – 5th chapter – Kushtha Nidana Adhyaya
- ✿ Madhava Nidana 49th chapter – Kushtha Nidana Adhyaya
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